FOLFOX-4

Supportive-care review sheet · Individualise with the team; not a prescription.

Before treatment

Confirm exact indication, current symptoms, allergies, blood counts, renal/liver function and medicines. Confirm DPD testing before fluorouracil; interpret uracil phenotyping cautiously with renal impairment.

Premedication and nausea

Antiemetics per NCCP/local moderate-risk protocol. moderate emetogenic risk; provide rescue instructions.

Administration

Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. For adjuvant FOLFOX-4, NCCP describes 12 cycles; metastatic treatment continues while suitable.

Skin, nerves and minerals

Assess functional neuropathy and cold symptoms. Avoid cold exposure/ice chips; oxaliplatin requires glucose-compatible lines and flushes.

Bowel symptoms

Give the written diarrhoea, hydration and escalation plan. Fever, blood, severe pain, reduced urine or inability to drink require urgent assessment.

Access and discharge

Confirm port/PICC care, pump/spill instructions and professional renewal/disconnection appointments. Disconnection is not removal of an implanted port.

Medication review

Review interactions and supplements with pharmacy. Check fluorouracil interactions including warfarin, phenytoin and DPD inhibitors. Discuss fertility, pregnancy, contraception and breastfeeding for the complete regimen.

Contact and follow-up

Give the guide, passport and contact card. Use teach-back for fever/illness, chest or breathing symptoms, severe bowel symptoms and line problems.

> Missing inputs remain unassessed; independent clinical/pharmacy review pending. FOLFOX4 table lists folinic acid on Day 1 only, while fluorouracil is Days 1 and 2. Confirm the Day 2 folinic-acid prescription; do not infer it from another protocol.

Sources: NCCP 00210 · NCCP antiemetic guidance